

1. Administrative & Study Identification

Full Study Title: A Phase I/Ib Global, Multicenter, Open-label Umbrella Study Evaluating the Safety and Efficacy of Targeted Therapies in Subpopulations of Patients With Metastatic Colorectal Cancer (INTRINSIC) **Short Title/Acronym:** INTRINSIC **Study Protocol Number:** W042758 **NCT Number:** NCT04929223 **Posting ID:** 897329816361833137 **Sponsor/Company Name:** Hoffmann-La Roche / Roche **Trial Status:** RECRUITING (Currently recruiting participants) **Investigational Product:** Targeted therapies and immunotherapies (Inavolisib, Bevacizumab, Cetuximab, Atezolizumab, Tiragolumab, SY-5609, Divarasib, FOLFOLX, FOLFIRI) **Phase of Development:** Phase 1/1b (Phase ID: PHASE1) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and efficacy (Objective Response Rate) of targeted therapies or immunotherapy as single agents or in combinations in participants with biomarker-positive metastatic colorectal cancer (mCRC). **Secondary Objectives:** To evaluate Duration of Response (DoR), Disease Control Rate (DCR), Progression-Free Survival (PFS), Overall Survival (OS), and Quality of Life measures across the respective treatment cohorts.

2.2 Background & Rationale To address the unmet need for precision medicine in advanced oncology by evaluating tailored targeted therapies and immunotherapies in specific subpopulations of patients with metastatic colorectal cancer (mCRC) whose tumors are biomarker positive (e.g., PIK3CA, KRAS G12C). This open-label umbrella study design efficiently matches genetic/biomarker profiles to experimental treatment arms to maximize patient benefit and drug efficacy signals.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Umbrella Master Protocol) **Control Type:** Single-arm per biomarker-specific cohort **Blinding:** Open-label **Randomization:** N/A (Assigned based on biomarker assay results)

3.2 Planned Enrollment & Duration Target \$N\$: 542 participants **Number of Sites:** Global Multicenter **Estimated Study Start:** 22-Oct-2021 **Estimated Primary Completion:** 31-Aug-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed metastatic colorectal cancer (mCRC). 3. Biomarker eligibility determined by a validated test approved by local health authorities or a CAP/CLIA-certified diagnostic laboratory (e.g., FoundationOne Liquid CDx). 4. Measurable disease according to RECIST v1.1 and baseline tumor tissue available for exploratory research.

4.2 Exclusion Criteria 1. Current participation or enrollment in another interventional clinical trial (follow-up periods of prior trials are permitted). 2. Any systemic anti-cancer treatment within 2 weeks or 5 half-lives (whichever is shorter) prior to the start of study treatment. 3. Pregnant or breastfeeding, or intending to become pregnant during the study. 4. Positive HIV test at screening (exception: eligible if stable on anti-retroviral therapy for at least 4 weeks, CD4 count ≥ 200 uL, undetectable viral load, and no AIDS-attributable opportunistic infections in the last 12 months).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Arm-specific based on matched biomarker. Examples include Inavolisib (9 mg PO QD), Cetuximab (400 mg/m² initial IV, then 250 mg/m² weekly), Divarasisb (PO QD), or other designated combinations on 28-day cycles. *Note on Bevacizumab:* Bevacizumab (ATC codes: S01LA08, L01FG01; Trade names: Avastin, Vegzelma, Alymsys, Zirabev, Mvasi). EMA Product Information available for [Alymsys](#), [Avastin](#), [Mvasi](#), [Vegzelma](#), and [Zirabev](#). **Route of Administration:** Oral (PO) and Intravenous (IV). **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent (Cycle length = 28 days).

5.2 Concomitant & Prohibited Therapy Permitted: Stable anti-retroviral therapies (for eligible HIV+ participants) and standard supportive care medications. **Prohibited:** Concurrent systemic anti-cancer treatments and other investigational therapies (washout required 28 days prior to initiation).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Biomarker Testing (CAP/CLIA), HIV Test, Baseline RECIST Tumor Scans
Baseline	Day 1 (Cycle 1)	Assignment to Treatment Arm, First Dose, Safety Labs, Vitals
Interim	Every 28 Days	Treatment Administration, Safety Labs, Efficacy Assessment (RECIST v1.1)
End of Study	Approx 84 Months	Final Tumor Assessment, Survival Follow-Up, Safety Exit Evaluation

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Confirmed Objective Response Rate (cORR) and incidence of adverse events/safety profile in specific biomarker subpopulations. **Measurement Method:** Investigator assessment according to Response Evaluation Criteria in Solid Tumors (RECIST) v1.1.

7.2 Secondary & Exploratory Endpoints 1. Duration of Response (DoR) and Disease Control Rate (DCR). 2. Progression-Free Survival (PFS), Overall Survival (OS), and Quality of Life (QoL) measures.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent and immune-related adverse events via clinical evaluation and laboratory tests. **Serious Adverse Events (SAEs):** Reported within 24 hours; dose interruptions or reductions implemented per protocol guidelines for high-grade toxicities. **Data Monitoring Committee (DMC):** Internal Hoffmann-La Roche safety review team.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (tumor response) outcomes. Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target $N = 542$ sized to provide adequate statistical power to evaluate early safety and efficacy signals across multiple distinct parallel biomarker-driven cohorts. **Handling of Missing Data:** Standard oncology methods including censoring for time-to-event endpoints (e.g., PFS, OS) at the time of the last valid assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Global multicenter monitoring to ensure strict protocol adherence and proper biomarker allocation. **Audit Trail:** eCRF locking and stringent central/local diagnostic test validation matching CAP/CLIA or FoundationOne testing requirements.

11. Study Identifiers & Governance

Primary ID: W042758 **Registry Number:** NCT04929223 **Posting ID:** 897329816361833137 **Sponsor/Lead Organization:** Hoffmann-La Roche / Roche **Study Title:** INTRINSIC Study **Official Regulatory Title:** A Phase I/Ib Global, Multicenter, Open-label Umbrella Study Evaluating the Safety and Efficacy of Targeted Therapies in Subpopulations of Patients With Metastatic Colorectal Cancer (INTRINSIC) **Criteria Type Requirement:** PHASE_REQUIREMENT (Trial must be in PHASE1) **Source Level:** 5

12. Clinical Framework (Metastatic Colorectal Cancer Specific)

Primary Condition (Disease Area): Metastatic Colorectal Cancer (mCRC) **Preferred Name:** Metastasis from malignant neoplasm of colon and/or rectum **Diagnosis Threshold:** Confirmed mCRC with positive validated biomarker assay result **Medical Methodology:** Precision medicine / Biomarker-guided Targeted Therapy **Optimization Target:** Objective tumor response / Progression-Free Survival

13. Study Procedures & Methodology

Management Pathway: Umbrella trial pathway matching genomic mutations to tailored investigational arms **Education Protocol (HCP):** Protocol training on distinct arm toxicities and RECIST measurement **Education Protocol (Patient):** Informed consent detailing genomic profiling and specific side effects **Quality Audit Mechanism:** Validation tracking of local CAP/CLIA diagnostic testing vs. central assays

14. Observation & Follow-Up Schedule

Total Duration: Estimated 84 months **Onsite Visit Cadence:** Every 28-day cycle **Remote Monitoring Frequency:** As needed for safety/survival follow-up **Checklist Review Interval:** Cycle-by-cycle tumor assessment according to RECIST v1.1

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	____	[DD-MMM-YYYY]				